

إسم المريض ربيعاً : انتصار خليفه حسين محمد
المحافظة : القاهرة الحي / المدينة : الساحل
رقم البطاقة : 26610090104269 رقم التليفون : 01027854769
العمر(تاريخ الميلاد) : 09-10-1966 المهنة : ربه منزل
العنوان : الساحل
تاريخ تسجيل الإستمارة : 23-09-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التعليق على التشخيص : -----

GR1-E

جوسٹاف
روسی
لیدی
جوسٹاف
روسی

صحة الشباب والمرفحة
اللجنة ومدير

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناقشة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

ملاحظات : ---

**رئيس
المجلس الطبي المتخصص**

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Name: انتصار خليفه حسين محمد National ID: 26610090104269	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

3- Medical number:
Female

83169556
Old Med :

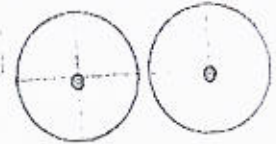
Old Med :
4022/2023

A-Patient ID:
1- Name : النصار خليفة حسين محمد

Sex:

Patient Age : 57 Y

2-ID number: 26610090104269



Family history

Family history

Phone

Phone
0102255257

BMI

BMI

REASON OF MDT

REASON OF MDT

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP
B

Date of MDT

Date of MDT
2023-11-23

Co morbidity

Co morbidity
HTN

Menopausal
status

Menopausal Status

Initial STAGE

Initial STAGE
Early Detect

Clinical data

Breast Imaging :

Free margins : VE LNS

Mets Workup :

BX

ER

PR

Her 2 Neu
1

SISH

Ki67
29%

post RT breast
BCSTALND for
invasive duct carcinoma

ER COMMENT
+ VE

PR COMMENT
+ VF

Her 2 Neu COMMENT

SISH COMMENT

T

PT1

N

PNO

M

0

Sonomammography

PATIENT HISTORY

US WAS DONE SHOWING PO OPERATIVE CHANGES

MDT OUTCOME & PLAN

IN VIEW OF HIGH GRADE
FOR ADJUVANT TAXOTERE /ENDOXAN X4 CYCLES THEN PORT
THEN RT (A1 FOR 5YEAR)

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition

MDT Form

[[DOCTYPE:MDT_FORM]]



Patient Name	: الأستاذ / انتصار خليفة حسين محمد	Request No.	: 13008653
Patient No.	: 26610090104269	Request Date	: 28/10/2023
Path No.	: SR-1308/23	Verification Date	: 13/11/2023
Sex	: Female	Financial Category	: نفقة الدولة - باب السلام
Age	: 57 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: marwa mohamed you

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast cancer, palpable right and left axillary lymph nodes.

Nature of Specimen: Slide Revision.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- E-Cadherin : Positive .
- ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8).
- PR: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8).
- Her2: Faint incomplete membrane reaction in the tumor cells (score 1+).
- Ki67: 29% nuclear reaction of tumor cells.

Conclusion:

Right breast cancer, palpable right and left axillary lymph nodes, Slide revision:

Invasive duct carcinoma Grade II.

Negative lymph nodes (0/21)

- Stage: pN0

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Score 1+.
- Ki67: Positive in 29% of tumor cells.

Verified by :





Patient Name	الأستاذة / انتصار خليفه حسين محمد	Request No.	13008653
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Referring Doc.		Registered by	.marwa mohamed yous

IMMUNOHISTOCHEMISTRY REPORT

Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Prof Dr. Lobna Omar
Dr. Menan Hossam

Verified by :

Printing Date : 19/12/2024 09:51:52

Page 2 of 2

Printed By : Mrs.Mayada Hegazy





Patient Name: Entesar Khalifa Hussein	Date : 19/12/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 58 Year(s)
Medical ID: 4022/2023	Pat. ID: 43858

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had CBS in the right breast.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- The breasts are almost entirely fatty (ACR B)
- No left breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- right breast LIQ parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast LIQ parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- No Bilateral breast solid or cystic mass lesions.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Right breast parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Yours truly

Signature Dr. Engy Abdallah

19/12/2024 5:06 PM



Patient Name: Entesar Khalifa Hussein	Date : 05/11/2022
Exam : C.T CHEST WITH CONTRAST	Age : 87 Year(s)
Medical ID:	Pat. ID: 17858

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

1. Chest:

- Right breast edema with skin thickening.
- Left axillary heterogeneously enhancing lesion measuring 7 cm.
- Clear both lungs.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.



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تعاليل وفضولات
الكرهون ~~فيلسوف~~ فيلساف
اوصيا